NCCP 00483 · VERSION 13A

Your treatment, explained

Nivolumab · every 2 weeks · previously treated NSCLC

14-day cycle | Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact the oncology team immediately for new or worsening breathlessness, chest pain, persistent diarrhoea, severe tummy pain, yellow skin/eyes, much less urine, severe headache, marked weakness, confusion, unusual thirst or passing much more urine, or a spreading/blistering rash. For severe breathing difficulty, collapse, seizure or other immediately dangerous symptoms, seek emergency care. Do not wait for the next infusion.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: hse · nccp · smpc

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
Your treatment and reviews
Your 14-day timeline
Baseline and cycle-specific checks
Whole treatment, scans and checks
Tablets, visits and practical planning
Your cancer and treatment aim
How nivolumab works
Your treatment course
Tests and review
Practical support
When to get help
Supportive medicines
Understanding the evidence
Questions to ask
Sources

UNDERSTAND YOUR TREATMENT

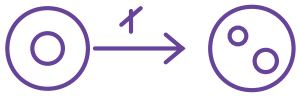
How nivolumab works

Aim to control advanced lung cancer, relieve symptoms and prolong life where possible.

IMMUNOTHERAPY

Nivolumab

Opdivo



Blocks PD-1 on immune cells

Nivolumab blocks PD-1 on immune cells, helping them recognise and attack cancer. It can also cause inflammation in healthy organs.

Immune cell → PD-1 blocked → Cancer attacked

What to watch for
Report new breathing, bowel, skin, hormone, heart or nerve symptoms promptly; these may be immune inflammation.

Sources: smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

THE BIGGER TREATMENT PLAN

Your treatment and reviews

REPEAT TREATMENT AND REVIEW

Nivolumab monotherapy

Cycle 1 | Cycle 2 | Cycle 3 | …

Nivolumab 240 mg IV over 30 minutes OR 600 mg as a subcutaneous injection over 3–5 minutes on Day 1 every 2 weeks. Your team confirms one formulation; the whole visit includes checks and preparation. Continue until progression or unacceptable toxicity, with individual benefit and safety review.

There is no planned chemotherapy induction or maintenance switch in this monotherapy pathway. Treatment is reviewed regularly and may be held, stopped or changed.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Checks across your treatment

This is a relative treatment map, not a booking calendar. Ask who books each test and reviews the result. Extra tests may be needed for symptoms.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00483 v13a · official regimen

THE WHOLE TREATMENT JOURNEY

Nivolumab · every 2 weeks · previously treated NSCLC: treatment and checks

Previously treated locally advanced or metastatic non-small-cell lung cancer (NSCLC).

Nivolumab 240 mg IV over 30 minutes OR 600 mg as a subcutaneous injection over 3–5 minutes on Day 1 every 2 weeks. Your team confirms one formulation; the whole visit includes checks and preparation. Continue until progression or unacceptable toxicity, with individual benefit and safety review.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	Between treatments	Response review	After treatment
Blood count, kidneys, liver and glucose NCCP 00483 · SMPC	Baseline tests	Results checked before treatment	Before every 2-week cycle	Continue during holds as directed	Follow unresolved abnormalities
Thyroid and hormones NCCP 00483 · SMPC	Baseline thyroid	Review symptoms/ medicines	Thyroid once a month; cortisol if indicated	Replacement/steroid plan reviewed	Continue endocrine care when needed
Symptoms and immune effects NCCP 00483 · SMPC	Record existing symptoms	Report reactions immediately	Report new symptoms any day	Clinical hold/restart review	Monitor at least 5 months after last dose
Cancer response NCCP 00483 · SMPC	Confirm previous platinum treatment	Agree review plan	Scans as consultant directs	Progression review before cycle 8	Agree follow-up/next treatment

What each check is for

1

Eligibility

Before first dose

Verify source indication

Ask / act: Confirm prescription, consent, baseline tests and alert card.

Sources: NCCP 00483 · SMPC

2

Response review

Before cycle 8

Assess progression

Ask / act: Document the response and continuation decision.

Sources: NCCP 00483 · SMPC

3

After final dose

At least 5 months

Immune effects can be delayed

Ask / act: Provide ongoing symptom/urgent contact and hormone-care plan.

Sources: NCCP 00483 · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-29 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.27

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Nivolumab 240 mg IV over 30 minutes OR 600 mg as a subcutaneous injection over 3–5 minutes on Day 1 every 2 weeks. Your team confirms one formulation; the whole visit includes checks and preparation. Continue until progression or unacceptable toxicity, with individual benefit and safety review.

Treatment timing: displayed days 1–14

Treatment or check	Days in this window	What happens
Treatment	Day 1	Day 1 · prescribed infusion or injection
Immune effects	Days 1–14	Any day · also after treatment ends
Energy, skin and bowels	Days 1–14	Timing varies · report changes promptly

Using the map: A relative treatment map, not a booking calendar.

Treatment duration: Continue until progression or unacceptable toxicity, with individual benefit and safety review.

Symptoms: Immune effects can begin during treatment or after it stops.

> **Why these bands stay broad:** Days show a typical interval between treatments; appointments can change after clinical review. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

> **There are no guaranteed "safe days".** Immune effects can happen on any day and after treatment ends. Carry your immunotherapy alert card and tell any healthcare team that you receive nivolumab.

Complete timing and symptom notes

Day 1: Your treatment visit

The team checks blood results, symptoms and medicines before the prescribed infusion or injection. Tell the nurse immediately about itching, flushing, breathlessness, dizziness or feeling suddenly unwell. Before leaving, confirm the next tests, treatment visit and urgent contact number.

Day 2–14: Between visits

You may feel well between infusions, but immune inflammation can begin at any time. Keep a short symptom record and take medicines exactly as prescribed. Contact your team about new symptoms instead of waiting for your next appointment. Confirm monthly thyroid tests and who arranges them; report new symptoms without waiting for the next treatment.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Appointments

Nivolumab 240 mg IV over 30 minutes OR 600 mg as a subcutaneous injection over 3–5 minutes on Day 1 every 2 weeks. Your team confirms one formulation; the whole visit includes checks and preparation.

STEP 2

Course length

Continue until progression or unacceptable toxicity, with individual benefit and safety review.

STEP 3

Tests and review

Blood count, kidney/liver tests and glucose before every 2-week cycle; thyroid once a month; cortisol if clinically indicated. Review cancer progression before cycle 8. Continue immune-toxicity monitoring for at least 5 months after the last dose.

STEP 4

Home records

Carry your immune alert card, medicine list and urgent contact details. Follow prescribed steroids or hormone replacement carefully; do not stop steroids suddenly.

STEP 5

After treatment

Keep reporting symptoms and attend agreed follow-up for at least 5 months after the final dose; some hormone replacement continues longer.

Sources: NCCP 00483 v13a · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Sickness medicines

Nivolumab alone has minimal sickness risk. Routine preventive anti-sickness medicines or premedication are not usually required; the team may prescribe symptom relief.

2 Immune alert card

Carry your alert card during treatment and afterwards. Show it when seeking urgent care; symptoms can be mistaken for an infection or an ordinary stomach upset.

3 Hormones and steroids

Some immune effects need steroids or hormone replacement. Follow the prescribed plan and do not stop steroids suddenly. Ask for help if symptoms worsen or you cannot take essential medicines. Hormone replacement alone does not always mean nivolumab must stop. Your team checks symptoms and recovery and reviews any medicines that suppress the immune system before deciding whether treatment can continue or restart.

4 Pregnancy and breastfeeding

Discuss pregnancy, fertility and breastfeeding before treatment. Use effective contraception during treatment and for at least 5 months after the last dose. Breastfeeding requires a specialist decision; contact your team promptly if pregnancy occurs.

Sources: nccp · smpc

TREATMENT REFERENCE · 01

Your cancer and treatment aim

Previously treated locally advanced or metastatic non-small-cell lung cancer (NSCLC).

Eligibility

Adults with locally advanced or metastatic NSCLC after prior chemotherapy; recurrence or progression during or after one prior platinum-containing doublet for advanced/metastatic disease; ECOG 0–1.

Sources: nccp · smpc

Treatment aim

Aim to control advanced lung cancer, relieve symptoms and prolong life where possible.

Sources: nccp · smpc

Choices

Discuss other treatment, trials and symptom-focused care with your team.

Sources: nccp · smpc

TREATMENT REFERENCE · 02

How nivolumab works

Immunotherapy supports immune attack.

PD-1 blockade

Nivolumab blocks PD-1 on immune cells, helping them recognise and attack cancer. It can also cause the immune system to inflame healthy organs.

Sources: nccp · smpc

Healthy organs

Inflammation can affect lungs, bowel, liver, kidneys, skin, glands, muscles, heart and nerves. Some effects are serious, delayed or permanent.

Sources: nccp · smpc

TREATMENT REFERENCE · 03

Your treatment course

Confirm the formulation with your team.

Administration

Nivolumab 240 mg IV over 30 minutes OR 600 mg as a subcutaneous injection over 3–5 minutes on Day 1 every 2 weeks. Your team confirms one formulation; the whole visit includes checks and preparation.

Sources: nccp · smpc

Course length

Continue until progression or unacceptable toxicity, with individual benefit and safety review.

Sources: nccp · smpc

Pauses

Treatment may be held or stopped for side effects. Nivolumab is not routinely dose-reduced. Tell the team about missed appointments.

Sources: nccp · smpc

TREATMENT REFERENCE · 04

Tests and review

New symptoms matter even when recent tests were normal.

Before starting

Blood count, kidney/liver tests, glucose and thyroid tests establish a baseline; discuss existing illness and all medicines.

Sources: nccp · smpc

During treatment

Blood count, kidney/liver tests and glucose before every 2-week cycle; thyroid once a month; cortisol if clinically indicated. Review cancer progression before cycle 8. Continue immune-toxicity monitoring for at least 5 months after the last dose.

Sources: nccp · smpc

Response review

Your team assesses progression before cycle 8; later scans follow the consultant plan.

Sources: nccp · smpc

TREATMENT REFERENCE · 05

Practical support

Tell your team what matters to you.

Energy and activity

Balance activity and rest. New marked fatigue with dizziness, headache, thirst or weakness needs assessment.

Sources: nccp · smpc

Medicines and pregnancy

Bring all medicine/supplement details. Discuss fertility and breastfeeding. Use effective contraception during treatment and for at least 5 months after the final dose.

Sources: nccp · smpc

Sickness

Minimal sickness risk: routine preventive sickness medicines are not usually required. Your team may prescribe symptom relief.

Sources: nccp · smpc

TREATMENT REFERENCE · 06

When to get help

Use your oncology urgent contact plan.

Report promptly

Report new breathlessness, persistent diarrhoea, severe pain, yellow eyes, reduced urine, severe headache, weakness, unusual thirst or a spreading/blistering rash immediately. Use your oncology contact plan; severe breathing difficulty, collapse, seizure or severe chest pain needs emergency care.

Sources: nccp · smpc

During treatment

Tell the nurse immediately about a reaction during the infusion or injection.

Sources: nccp · smpc

After treatment

Continue monitoring for at least 5 months after the final dose. Show your immune alert card to anyone treating you.

Sources: nccp · smpc

Why this treatment may be offered

Nivolumab can help the immune system control some previously treated lung cancers. Some people benefit for a long time; others do not benefit.

Trial results describe groups, not an individual prognosis.

Sources: nccp · smpc · checkmate

PREPARE FOR THE CONVERSATION

Questions worth asking

Why am I having nivolumab?

Previously treated locally advanced or metastatic non-small-cell lung cancer (NSCLC). It is used after previous chemotherapy to help control cancer; ask how the benefits and alternatives apply to you.

Which formulation and schedule will I receive?

Nivolumab 240 mg IV over 30 minutes OR 600 mg as a subcutaneous injection over 3–5 minutes on Day 1 every 2 weeks. Your team confirms one formulation; the whole visit includes checks and preparation.

How long will treatment continue?

Continue until progression or unacceptable toxicity, with individual benefit and safety review.

Who arranges my tests and scans?

Blood count, kidney/liver tests and glucose before every 2-week cycle; thyroid once a month; cortisol if clinically indicated. Review cancer progression before cycle 8. Continue immune-toxicity monitoring for at least 5 months after the last dose.

When should I call the team?

Report new breathlessness, persistent diarrhoea, severe pain, yellow eyes, reduced urine, severe headache, weakness, unusual thirst or a spreading/blistering rash immediately. Use your oncology contact plan; severe breathing difficulty, collapse, seizure or severe chest pain needs emergency care.

What are my other options?

Ask about other anticancer treatment, trials and symptom-focused care suited to your cancer and preferences. Support remains available if you decline treatment.

> **Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00483 v13a. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00483 v13a · official regimen
- EMA Opdivo current product information
- CheckMate 017/057 five-year pooled follow-up, Borghaei 2021
- HSE · chemotherapy safety and when to contact your team
- NCCP NCIS medical-oncology antiemetic guidance v8,2025

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-10-29.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6498/483_Nivolumab_14-day_.pdf

https://www.ema.europa.eu/en/documents/product-information/opdivo-epar-product-information_en.pdf

https://pmc.ncbi.nlm.nih.gov/articles/PMC8078445/

https://www2.hse.ie/conditions/chemotherapy/side-effects/

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf